

PTCA REPORT

Consultant:Dr. HariprasadI DNB (Cardiology)

Name: SUDHEESAN	Age:47 Y Sex: MALE	Date: 31-08-2026
CATH NO 24988		IP No:607066
CATH TECH : JOSHY, SIDAN		Scrub nurse: JINCY

Premedication : Nil

Vascular Access : RRA

Target Vessel/lesions : LAD (80-90%)

Inventory for PTCA to LAD

Sheath : 6F Radifocus terumo

Guiding Catheter : 6F EBU 3.5

Guide wire : 0.014" Runthrough floppy

Pre dilatation balloon : 2.0x20mm minitek @ 12,14,14atm

: 2.5x12 Accuforce NC@ 12,14atm

Stent :2.75x28mm Xience Alpine @ 10atm (32.81)

Post dilatation balloon : 2.75x12mm Accuforce NC @ 14,20,20,14atm

Result : Good

Complications : Nil

Adjuvants : Heparin 7000units

Contrast : 100ml Omniaque

Hemodynamic Data : Aortic: Pre PTCA 120/70mm Hg

PROCEDURE:

Patient was taken up for PTCA with informed consent, RRA access was taken Later LCA was cannulated with a 6F EBU 3.5guiding Catheter. And the LAD lesion was crossed with a 0.014” Runthrough floppy guide wire and the lesion was predilated sequentially with a 2x20mm minitrek balloon at, 12,14,14atm and 2.5x12mm Accuforce NC @ 12,14atm. Later a **2.75x28mm Xience alpine** stent was deployed to the proximal to mid LAD at 10atm to a size of 2.81mm. The proximal, mid, distal part of the stent was post dilated with a 2.75x12mm Accuforce NC balloon at 14,20,20,14atm. Check shoots revealed well deployed stent with no residual stenosis, no thrombus, no dissection with good vessel flow distally. There was no procedure related complications.

COMMENT: PTCA WITH STENTING OF LAD (1DES) WAS DONE SUCCESSFULLY

Dr. HariprasadI DNB (Cardiology)
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